Everolimus · neuroendocrine tumours

Your neuroendocrine tumour treatment Progressive unresectable or metastatic pancreatic neuroendocrine tumour (NET), or well-differentiated G1/G2 nonfunctional gastrointestinal NET under NCCP 00320b/c.

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Continuous daily everolimus · neuroendocrine tumour (NET)

Active medicines: Everolimus

Emetogenic risk: Oral minimal-to-low

No routine prophylactic antiemetic order is populated for this phase. Agree an individual symptom-treatment plan where needed.

Additional phase-specific support

- Use gentle mouth care and any prescribed alcohol-free mouthwash. Ask before buying a rinse; avoid products containing alcohol, peroxide, iodine or thyme. Report ulcers early.
- Everolimus has minimal-to-low emetogenic potential. Routine premedication is not required by this regimen; agree any rescue anti-sickness or diarrhoea medicine with the team.
- Existing infection must be assessed and resolved before starting. Hepatitis B testing informs any specialist prevention plan. Vaccination and any antimicrobial prevention, including Pneumocystis jirovecii pneumonia (PJP) prevention with certain steroid plans, are individual decisions.
- Report weight loss, poor intake or persistent fatigue. Ask for nutritional and symptom support; do not continue through severe symptoms simply to finish a pack.
- No routine premedication is required by National Cancer Control Programme (NCCP). No universal steroid, antimicrobial or growth-factor prescription is implied. Mouthwash and rescue treatment follow the individual prescription.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

Sources checked 2026-11-05 · SACTCheck v0.84.37 · Regimen-level drafts; prescriber and oncology-pharmacy verification required